

Patient Messaging Tool — Overview

23 September 2026 · Ian Moore · working draft, updated after your email

What this is

A private channel between you and your post-op patients, run from your clinic. Patients check in by picking their procedure from a list and ticking boxes, and a running summary sits above each conversation, so when a message arrives weeks later you know who this is and what you already told them without rereading anything.

Two pages, five minutes. There is a longer version with the privacy architecture and the technical detail if you ever want it, but nothing in it needs you.

Updated 23 September after your email: post-op patients first, check-ins by dropdown and checkbox, and nothing from the hospital chart or ConnectingOntario. It is still a draft, so tell me what is wrong with it before I build anything.

What the first version does

- **Post-op check-ins.** The patient picks their procedure from a list of the ones you perform, then answers a short set of questions by ticking boxes. No essays, since many patients struggle to describe symptoms in words.
- **Red-flag rules** that pull a check-in or message to the top when it matches something on a list you write. Fixed rules, not judgement — nothing decides on its own that a patient is fine.
- **Messages** between you and a patient, in one thread, in an app on your iPad or phone rather than a web page.
- **The summary above the thread**, rewritten every time something new arrives, so it is never stale.
- **Still-open items** — questions you asked that were never answered, and questions the patient asked that you have not answered yet. This is the part I expect to earn its keep.
- **Saved replies** for the questions that come up constantly, editable before you send. Probably the biggest time saving in the whole thing, and the least clever.
- **The procedure and its date, set once.** The patient picks them when they join; you confirm them and add when you took them on. The post-op day follows automatically.
- **Calls and visits** logged with one tap, so the contact record covers what happens off the app.

Nothing is written by AI on your behalf, and nothing sends without you. The model's only job in this version is summarising the conversation.

One thing to settle on the phone: patients meet the checkboxes first, but should they also be able to type a message in their own words? My plan has a message box for both of you; your email leans towards checkboxes only for patients.

What you would see

Three cards follow, one to a page, each with a message that has just arrived from a patient you last spoke to some weeks ago. Cards A and B are the same post-op patient, in full and then collapsed; Card C is a different patient, six days before surgery. The patients and timelines are invented, and my notes on all three come after them.

The cards were drawn before your email, so they show messages but not the check-in yet. In the first version, the patient's latest check-in answers sit at the top of the card, above the summary, with any red-flag answer highlighted.

Margaret Chen

Laparoscopic right hemicolectomy · 14 Aug 2026

4 mo 21 d
IN YOUR CARE

19
POST-OP DAY

apixaban 5 mg BID

pathology pending · day 8

⚠ FLAGGED

today 09:14

“the redness is bigger today and i felt hot last night, didn’t take my temperature”

spreading redness

subjective fever

on anticoagulant

CONVERSATION SUMMARY

inferred · every line opens its source

SHE REPORTED

day 12 redness at lower end of incision [3]

day 15 loose stools, 4–5× daily [7]

day 18 wound “looks angrier” [11]

YOU ADVISED

day 12 keep it dry; watch for fever or spreading [4]

day 15 expected at this stage; keep fluids up [8]

STILL OPEN

day 12 you asked for her temperature if the redness spread — never answered [4]

day 18 she asked when she can drive again [11]

RECURRING wound appearance — 3 mentions (day 12, 18, 19)

LATEST MESSAGES

counted

You day 18 · 17:40 ✓ seen 18:05

Keep the dressing dry and send me a photo if it changes again.

Margaret day 18 · 16:05 [11]

“the wound looks angrier today. also when can i start driving?”

You day 15 · 11:20 ✓ seen 12:05

That’s expected at this stage — keep your fluids up.

CONTACT

counted

12

messages · 7 her / 5 you

1

clinic visit · day 8

1

call · 6 min, day 13

day 5

first contact

2 d

since last contact

1

awaiting your reply

YOUR TIME ON THIS PATIENT

18 min in app, across 9 sessions

measured

6 min one logged call

entered

24 min total since 19 Aug

median reply 4 h 10 m

Coverage: messages counted automatically · calls and visits entered by you

Saved reply

Log a call

Full thread (12)

Card B — post-op, collapsed (my proposed default)

Margaret Chen

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4 mo 21 d
IN YOUR CARE

19
POST-OP DAY

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 **FLAGGED**

today 09:14

“the redness is bigger today and i felt hot last night, didn’t take my temperature”

spreading redness

subjective fever

on anticoagulant

STILL OPEN

day 12 you asked for her temperature if the redness spread — never answered [4]

day 18 she asked when she can drive again [11]

▼ Full summary — 3 reported, 2 advised, wound mentioned 3×

inferred

LATEST MESSAGES

counted

You day 18 · 17:40

✓ seen 18:05

Keep the dressing dry and send me a photo if it changes again.

Margaret day 18 · 16:05

[11]

“the wound looks angrier today. also when can i start driving?”

12 msg 1 visit 1 call last 2 d ago

24 min

counted

Saved reply

Log a call

Full thread (12)

Joseph Okafor

Laparoscopic sigmoid colectomy · booked 27 Sep 2026

3 mo 16 d
IN YOUR CARE

6
DAYS TO SURGERY

warfarin · bridging plan sent day 38

CT staging complete

⚠️ FLAGGED

today 07:52

“the prep sheet says clear fluids from noon on the 26th but i take my blood thinner at 6pm — do i still take it that day?”

anticoagulant before surgery

6 days to surgery

CONVERSATION SUMMARY

inferred · every line opens its source

HE ASKED

–19 d whether he can keep taking his iron tablets [2]

–11 d what time to arrive and who can drive him home [5]

YOU ADVISED

–19 d stop the iron a week before; confirm when done [3]

–11 d arrival time sent; escort required, no taxi alone [6]

STILL OPEN

–19 d you asked him to confirm he had stopped the iron — message never opened [3]

LATEST MESSAGES

counted

You –11 d · 14:10

✓ seen 19:30

Arrive at 06:30 on the day. You will need someone to take you home — not a taxi on your own.

Joseph –11 d · 13:55

[5]

“what time should i get there, and can i just take a cab home?”

You –19 d · 09:15

✓ never opened

Stop the iron tablets a week before surgery. Let me know once you have stopped.

7 msg 2 clinic 0 calls last 11 d ago

11 min counted

Saved reply

Log a call

Full thread (7)

Reading the cards

Card A, and why it saves the reread. You have read nothing and you already know this is the second wound complaint, that you asked for a temperature a week ago and never got one, and that she is anticoagulated.

The parts in typewriter figures are counted — days, message totals, minutes — and are exactly right. The rest is written by a model from the conversation, and every line of it opens the message it came from, so you can check any of it in a tap.

Card B is the same information with everything non-urgent folded away, and is what I think should be on screen by default. Whether A or B is right is genuinely your call, and it is the first thing I would like to be wrong about. If it helps, tell me by card letter — they are labelled the same way in the longer version.

Card C: before surgery the clock runs the other way. Post-op day does not exist yet, so the badge counts down instead, and the summary is measured in days before the operation. Post-op comes first, because that is where the calls are; this card follows once that works.

The two badges say different things. The dark one is the clinical clock — post-op day, or a countdown before surgery. The pale one beside it is how long this person has been yours in total, which for Margaret is nearly five months against a post-op day of nineteen. Nothing else on the card tells you that, and it changes the tone of a reply.

It counts from when the patient came into your care, not from their first message in the app. Someone you have followed for two years who joined last month should not read as a new patient.

The last few messages sit below the summary, with read receipts on yours. Two ticks means the patient opened it; one tick means it was delivered and they have not. Look at Card C for why that matters: the instruction to stop the iron tablets was never opened, which is why he never confirmed. “He has not replied” and “he never saw it” call for completely different responses, and only one of them is a phone call.

Whether you want patients to see read receipts on your side is a separate question, and one I would rather you decided — knowing you have read a message and not yet replied sets an expectation.

Later on there is a third case — surveillance patients years out from surgery — where neither clock makes sense and it would just show how long they have been under your care.

What comes after

Your own clinic notes, never the hospital’s. Nothing from the hospital chart or ConnectingOntario goes into the app, as you said. If the first version shows you often need more than the conversation to answer, the next step draws on your own Accuro notes: read on your own device, turned into a short list of facts, and shown to you to confirm before any of it is kept.

Wound photos. A patient attaches a photo to a check-in, taken inside the app so it never sits in their camera roll. It is the most sensitive thing a patient can send, so it comes once the basics have proved themselves, with its own consent and storage rules.

Drafted replies. The tool writes a first draft from your own protocols for you to edit. It is last, because you have not asked for it, because saved replies probably absorb most of it, and because it carries more risk than everything else combined.

None of these is needed for the first version to be worth using.

Then your colleagues, and other clinics. Once it works for you, the partners at your clinic, then other outpatient clinics. If it ever goes to the hospital, the pitch is the one you named: fewer emergency department visits.

The four stages, at a glance

Each stage is built only if the one before it earned it, so the later rows may never happen.

Stage	What it is	What you would do	Uses real patient data?
v0 — sketches	Clickable mockups of the check-in and the cards, on invented patients. No app, nothing installed, nothing real	Tell me what is wrong with them	No. Invented patients only
v1 — post-op check-ins and messaging	The first version you would actually use: check-ins by dropdown and checkbox, messages, the rolling summary, still-open items, saved replies, red-flag rules, the contact and time card	Use it with a handful of post-op patients and tell me whether it cuts your calls and follow-up visits	Yes: what your patients tell you themselves. Needs their consent and an agreement between your clinic and the company running the app, not the hospital
v2 — photos and your own notes	Wound photos from patients; your Accuro notes, read on your device, with you confirming the facts before any are kept	Confirm or correct what it extracts, and compare it against your own reading for a while before trusting it	Yes, and more sensitive: photos, and your own clinic notes. Never the hospital chart or ConnectingOntario
Later	Your colleagues, then other clinics; verifiable records of what was said and approved; AI-drafted replies; check-in reminders at set days after surgery; spotting patients who have gone quiet; the hospital, on emergency visits	Nothing, unless you ask for one of them	Depends which, and each gets its own conversation first

I am deliberately not giving you dates yet. With the hospital out of the loop, the pace is set by the build and by getting the consent wording and your clinic's agreement right, which I would rather do properly than quickly.

What I need from you

Six things. The first three are the ones that actually block me.

1. **The procedures you perform.** The list patients will pick from, named the way patients would recognise them.
2. **The check-in questions, and which answers jump the queue.** What you would ask a post-op patient on the phone, written as tick-box options, and which answers should always reach you first. This is the most safety-critical thing in the system and it can only come from you.
3. **A rough count before we start.** For a few weeks: post-op calls to the office, calls you return, and follow-up visits. A tally at the front desk is enough. Without it we cannot show the drop later.
4. **The questions you answer over and over.** These become the saved replies, and they are probably where most of your time back comes from.
5. **Mark up the cards.** What is missing, what you would throw away, and which of the two layouts you would rather have.
6. **What device would you use?** Your own iPad or phone, or one the clinic or hospital manages, and whether you can install an app on it.

No rush on any of it. The phone call is the easiest place to start on the first two, and the count takes a few weeks, so it is worth starting early.

Things you should know before you get enthusiastic

Nothing real happens until the privacy basics are in place. Your clinic is the legal custodian of its own patients' information, so this does not need the hospital. It does need patient consent, a written agreement between your clinic and the company running the app, and data kept in Canada. Check-ins and messages become part of the medical record, with the ten-year retention rule that comes with it.

This is not a billing tool. Ontario's secure-messaging pilot ended in March with nothing announced to replace it, and post-op communication by the operating surgeon may sit inside the global fee anyway. The tool records your time honestly, which is useful as evidence of unfunded work. It is not a claim you can submit, and you would need to confirm both of those with someone who knows better than I do.

It measures you as well as the patients. Time per patient, response times. That is yours — not shown to patients and not reported to the hospital — but you should know it exists rather than find it later.

Messaging changes what patients expect. Easier to reach you means more messages, and an expectation of quicker replies. The queue and the saved replies are meant to absorb that, but the honest risk is that it costs you more time than it gives back. That is the main way this fails, and it is worth deciding up front what response time you are willing to promise.

Keeping the hospital out means not asking them, not keeping it secret. If you are on the Mackenzie board, a short note disclosing the family interest protects you. If it surfaces later without one, that is what gives admin a real handle.

I am not a lawyer and none of this is legal advice. And to be plain about my side: I would like this to become something other surgeons pay for, and eventually a company. I also have a commercial interest in one optional piece further down the road, the service behind the verifiable records.

I would rather you tell me this is not worth it than politely use it twice. The whole point of showing you cards before code is that it is cheap to be wrong right now.